

COMMITTEE ON REGULATORY AFFAIRS AND GOVERNMENT EFFICIENCY
SENATE AMENDMENTS TO H.B. 2408
(Reference to House engrossed bill)

Amendment instruction key:

[GREEN UNDERLINING IN BRACKETS] indicates text added to statute or previously enacted session law.

[Green underlining in brackets] indicates text added to new session law or text restoring existing law.

~~[GREEN STRIKEOUT IN BRACKETS]~~ indicates new text removed from statute or previously enacted session law.

~~[Green strikeout in brackets]~~ indicates text removed from existing statute, previously enacted session law or new session law.

<<Green carets>> indicate a section added to the bill.

~~<<Green strikeout in carets>>~~ indicates a section removed from the bill.

1 The bill as proposed to be amended is reprinted as follows:

2 Section 1. Section 32-1601, Arizona Revised Statutes, is amended to
3 read:

4 32-1601. Definitions

5 In this chapter, unless the context otherwise requires:

6 1. "Absolute discharge from the sentence" means completion of any
7 sentence, including imprisonment, probation, parole, community supervision
8 or any form of court supervision.

9 2. "Appropriate health care professional" means a licensed health
10 care professional whose scope of practice, education, experience, training
11 and accreditation are appropriate for the situation or condition of the
12 patient who is the subject of a consultation or referral.

13 3. "Approval" means that a regulated training or educational
14 program to prepare persons for licensure, certification or registration
15 has met standards established by the board.

16 4. "Board" means the Arizona state board of nursing.

17 5. "Certified nurse midwife" means a registered nurse who:

18 (a) Is certified by the board.

19 (b) Has completed a nurse midwife education program THAT IS
20 approved or recognized by the board and educational requirements THAT ARE
21 prescribed by the board by rule.

22 (c) Holds a national certification as a certified nurse midwife
23 from a national certifying body THAT IS recognized by the board.

24 (d) Has an expanded scope of practice in providing health care
25 services for women from adolescence to beyond menopause, including
26 antepartum, intrapartum, postpartum, reproductive, gynecologic and primary
27 care, for normal newborns during the first twenty-eight days of life and

1 for men for the treatment of sexually transmitted diseases. The expanded
2 scope of practice under this subdivision includes:

3 (i) Assessing patients, synthesizing and analyzing data and
4 understanding and applying principles of health care at an advanced level.

5 (ii) Managing the physical and psychosocial health care of
6 patients.

7 (iii) Analyzing multiple sources of data, identifying alternative
8 possibilities as to the nature of a health care problem and selecting,
9 implementing and evaluating appropriate treatment.

10 (iv) Making independent decisions in solving complex patient care
11 problems.

12 (v) Diagnosing, performing diagnostic and therapeutic procedures
13 and prescribing, administering and dispensing therapeutic measures,
14 including legend drugs, medical devices and controlled substances, within
15 the scope of the certified nurse midwife practice after meeting
16 requirements established by the board.

17 (vi) Recognizing the limits of the ~~nurse's~~ CERTIFIED NURSE
18 MIDWIFE'S knowledge and experience by consulting with or referring
19 patients to other appropriate health care professionals if a situation or
20 condition occurs that is beyond the knowledge and experience of the
21 CERTIFIED nurse MIDWIFE or if the referral will protect the health and
22 welfare of the patient.

23 (vii) Delegating to a medical assistant pursuant to section
24 32-1456.

25 (viii) Performing additional acts that require education and
26 training as prescribed by the board and that are recognized by the nursing
27 profession as proper to be performed by a certified nurse midwife.

28 6. "Certified nursing assistant" means a person who is registered
29 on the registry of nursing assistants pursuant to this chapter to provide
30 or assist in delivering nursing or nursing-related services under the
31 supervision and direction of a licensed nursing staff member. Certified
32 nursing assistant does not include a person who:

33 (a) Is a licensed health care professional.

34 (b) Volunteers to provide nursing assistant services without
35 monetary compensation.

36 (c) Is a licensed nursing assistant.

37 7. "Certified registered nurse" means a registered nurse who has
38 been certified by a national nursing credentialing agency THAT IS
39 recognized by the board.

40 8. "Certified registered nurse anesthetist" means a registered
41 nurse who meets the requirements of section 32-1634.03 and who practices
42 pursuant to the requirements of section 32-1634.04.

43 9. "Clinical nurse specialist" means a registered nurse who:

44 (a) Is certified by the board as a clinical nurse specialist.

45 (b) Holds a graduate degree with a major in nursing and completes
46 educational requirements as prescribed by the board by rule.

1 (c) Is nationally certified as a clinical nurse specialist or, if
2 certification is not available, provides proof of competence to the board.

3 (d) Has an expanded scope of practice based on advanced education
4 in a clinical nursing specialty that includes:

5 (i) Assessing clients, synthesizing and analyzing data and
6 understanding and applying nursing principles at an advanced level.

7 (ii) Managing directly and indirectly a client's physical and
8 psychosocial health status.

9 (iii) Analyzing multiple sources of data, identifying alternative
10 possibilities as to the nature of a health care problem and selecting
11 appropriate nursing interventions.

12 (iv) Developing, planning and guiding programs of care for
13 populations of patients.

14 (v) Making independent nursing decisions to solve complex client
15 care problems.

16 (vi) Using research skills and acquiring and applying critical new
17 knowledge and technologies to nursing practice.

18 (vii) Prescribing and dispensing durable medical equipment.

19 (viii) Consulting with or referring a client to other health care
20 providers based on assessment of the client's health status and needs.

21 (ix) Facilitating collaboration with other disciplines to attain
22 the desired client outcome across the continuum of care.

23 (x) Performing additional acts that require education and training
24 as prescribed by the board and that are recognized by the nursing
25 profession as proper to be performed by a clinical nurse specialist.

26 (xi) Prescribing, ordering and dispensing pharmacological agents
27 THAT ARE subject to the requirements and limits specified in section
28 32-1651.

29 10. "Conditional license" or "conditional approval" means a license
30 or approval that specifies the conditions under which the regulated party
31 is allowed to practice or to operate and that is prescribed by the board
32 pursuant to section 32-1644 or 32-1663.

33 11. "Delegation" means transferring to a competent individual the
34 authority to perform a selected nursing task in a designated situation in
35 which the nurse making the delegation retains accountability for the
36 delegation.

37 12. "Disciplinary action" means a regulatory sanction of a license,
38 certificate or approval pursuant to this chapter in any combination of the
39 following:

40 (a) A civil penalty for each violation of this chapter, not to
41 exceed \$1,000 for each violation.

42 (b) Restitution made to an aggrieved party.

43 (c) A decree of censure.

44 (d) A conditional license or a conditional approval that ~~fixed~~
45 FIXES a period and terms of probation.

46 (e) Limited licensure.

47 (f) Suspension of a license, a certificate or an approval.

- 1 (g) Voluntary surrender of a license, a certificate or an approval.
2 (h) Revocation of a license, a certificate or an approval.
- 3 13. "Health care institution" has the same meaning prescribed in
4 section 36-401.
- 5 14. "Licensed health aide" means a person who:
6 (a) Is licensed pursuant to this chapter to provide or to assist in
7 providing nursing-related services authorized pursuant to section 36-2939.
8 (b) Is the parent, guardian or family member by affinity or
9 consanguinity of the Arizona long-term care system member receiving
10 services who may provide licensed health aide services only to that member
11 and only consistent with that member's plan of care.
12 (c) Has a scope of practice that is the same as a licensed nursing
13 assistant and may also provide medication administration, tracheostomy
14 care, enteral care and routine ventilator care and therapy and any other
15 tasks approved by the board in rule.
16 (d) Has supervision requirements that are the same as a certified
17 nursing assistant.
- 18 15. "Licensed nursing assistant" means a person who is licensed
19 pursuant to this chapter to provide or assist in delivering nursing or
20 nursing-related services under the supervision and direction of a licensed
21 nursing staff member. Licensed nursing assistant does not include a
22 person who:
23 (a) Is a licensed health care professional.
24 (b) Volunteers to provide nursing assistant services without
25 monetary compensation.
26 (c) Is a certified nursing assistant.
- 27 16. "Licensee" means a person who is licensed pursuant to this
28 chapter or in a party state as defined in section 32-1668.
- 29 17. "Limited license" means a license that restricts the scope or
30 setting of a licensee's practice.
- 31 18. "Medication order" means a written or verbal communication
32 given by a certified registered nurse anesthetist to a health care
33 professional to administer a drug or medication, including controlled
34 substances.
- 35 19. "Practical nurse" means a person who holds a practical nurse
36 license issued pursuant to this chapter or pursuant to a multistate
37 compact privilege and who practices practical nursing ~~as defined in this~~
38 ~~section~~.
- 39 20. "Practical nursing" includes the following activities that are
40 performed under the supervision of a physician or a registered nurse:
41 (a) Contributing to the assessment of the health status of
42 individuals and groups.
43 (b) Participating in the development and modification of the
44 strategy of care.
45 (c) Implementing aspects of the strategy of care within the nurse's
46 scope of practice.

1 (d) Maintaining safe and effective nursing care that is rendered
2 directly or indirectly.

3 (e) Participating in the evaluation of responses to interventions.

4 (f) Delegating nursing activities within the scope of practice of a
5 practical nurse.

6 (g) Performing additional acts that require education and training
7 as prescribed by the board and that are recognized by the nursing
8 profession as proper to be performed by a practical nurse.

9 21. "Presence" means within the same health care institution or
10 office as specified in section 32-1634.04, subsection A, ~~and~~ and available as
11 necessary.

12 22. "Registered nurse" or "professional nurse" means a person who
13 practices registered nursing and who holds a registered nurse license
14 issued pursuant to this chapter or pursuant to a multistate compact
15 privilege.

16 23. "Registered nurse practitioner" means a registered nurse who:

17 (a) Is certified by the board.

18 (b) Has completed a nurse practitioner education program **THAT IS**
19 approved or recognized by the board and educational requirements **THAT ARE**
20 prescribed by the board by rule.

21 (c) If applying for certification after July 1, 2004, holds
22 national certification as a nurse practitioner from a national certifying
23 body **THAT IS** recognized by the board.

24 (d) Has an expanded scope of practice within a specialty area that
25 includes:

26 (i) Assessing clients, synthesizing and analyzing data and
27 understanding and applying principles of health care at an advanced level.

28 (ii) Managing the physical and psychosocial health status of
29 patients.

30 (iii) Analyzing multiple sources of data, identifying alternative
31 possibilities as to the nature of a health care problem and selecting,
32 implementing and evaluating appropriate treatment.

33 (iv) Making independent decisions in solving complex patient care
34 problems.

35 (v) Diagnosing, performing diagnostic and therapeutic procedures,
36 and prescribing, administering and dispensing therapeutic measures,
37 including legend drugs, medical devices and controlled substances within
38 the scope of registered nurse practitioner practice on meeting the
39 requirements established by the board.

40 (vi) Recognizing the limits of the ~~nurse's~~ **REGISTERED NURSE**
41 **PRACTITIONER'S** knowledge and experience by consulting with or referring
42 patients to other appropriate health care professionals if a situation or
43 condition occurs that is beyond the knowledge and experience of the
44 **REGISTERED** nurse **PRACTITIONER** or if the referral will protect the health
45 and welfare of the patient.

46 (vii) Delegating to a medical assistant pursuant to section
47 32-1456.

- 1 (viii) Performing additional acts that require education and
2 training as prescribed by the board and that are recognized by the nursing
3 profession as proper to be performed by a REGISTERED nurse practitioner.
- 4 24. "Registered nursing" includes the following:
- 5 (a) Diagnosing and treating human responses to actual or potential
6 health problems.
- 7 (b) Assisting individuals and groups to maintain or attain optimal
8 health by implementing a strategy of care to accomplish defined goals and
9 evaluating responses to care and treatment.
- 10 (c) Assessing the health status of individuals and groups.
- 11 (d) Establishing a nursing diagnosis.
- 12 (e) Establishing goals to meet identified health care needs.
- 13 (f) Prescribing nursing interventions to implement a strategy of
14 care.
- 15 (g) Delegating nursing interventions to others who are qualified to
16 do so.
- 17 (h) Providing for the maintenance of safe and effective nursing
18 care that is rendered directly or indirectly.
- 19 (i) Evaluating responses to interventions.
- 20 (j) Teaching nursing knowledge and skills.
- 21 (k) Managing and supervising the practice of nursing.
- 22 (l) Consulting and coordinating with other health care
23 professionals in the management of health care.
- 24 (m) Performing additional acts that require education and training
25 as prescribed by the board and that are recognized by the nursing
26 profession as proper to be performed by a registered nurse.
- 27 25. "Registry of nursing assistants" means the nursing assistants
28 registry maintained by the board pursuant to the omnibus budget
29 reconciliation act of 1987 (P.L. 100-203; 101 Stat. 1330), as amended by
30 the medicare catastrophic coverage act of 1988 (P.L. 100-360; 102 Stat.
31 683).
- 32 26. "Regulated party" means any person or entity that is licensed,
33 certified, registered, recognized or approved pursuant to this chapter.
- 34 27. "Unprofessional conduct" includes the following, whether
35 occurring in this state or elsewhere:
- 36 (a) Committing fraud or deceit in obtaining, attempting to obtain
37 or renewing a license or a certificate issued pursuant to this chapter.
- 38 (b) Committing a felony, whether or not involving moral turpitude,
39 or a misdemeanor involving moral turpitude. In either case, conviction by
40 a court of competent jurisdiction or a plea of no contest is conclusive
41 evidence of the commission.
- 42 (c) Aiding or abetting in a criminal abortion or attempting,
43 agreeing or offering to procure or assist in a criminal abortion.

- 1 (d) COMMITTING any conduct or practice that is or might be harmful
2 or dangerous to the health of a patient or the public.
- 3 (e) Being mentally incompetent or physically unsafe to a degree
4 that is or might be harmful or dangerous to the health of a patient or the
5 public.
- 6 (f) Having a license, certificate, permit or registration to
7 practice a health care profession denied, suspended, conditioned, limited
8 or revoked in another jurisdiction and not reinstated by that
9 jurisdiction.
- 10 (g) Wilfully or repeatedly violating a provision of this chapter or
11 a rule adopted pursuant to this chapter.
- 12 (h) Committing an act that deceives, defrauds or harms the public.
- 13 (i) Failing to comply with a stipulated agreement, consent
14 agreement or board order.
- 15 (j) Violating this chapter or a rule that is adopted by the board
16 pursuant to this chapter.
- 17 (k) Failing to report to the board any evidence that a registered
18 ~~or~~ NURSE, practical nurse or ~~a~~ nursing assistant is or may be:
- 19 (i) Incompetent to practice.
- 20 (ii) Guilty of unprofessional conduct.
- 21 (iii) Mentally or physically unable to safely practice nursing or
22 to perform nursing-related duties. A nurse who is providing therapeutic
23 counseling for a nurse who is in a drug rehabilitation program is required
24 to report that nurse only if the nurse providing therapeutic counseling
25 has personal knowledge that patient safety is being jeopardized.
- 26 (l) Failing to self-report a conviction for a felony or
27 undesignated offense within ten days after the conviction.
- 28 (m) Cheating or assisting another to cheat on a licensure or
29 certification examination.
- 30 (n) ENGAGING IN SEXUAL CONDUCT WITH A CURRENT PATIENT OR WITH A
31 FORMER PATIENT WITHIN SIX MONTHS AFTER THE LAST TREATMENT UNLESS THE
32 PATIENT WAS THE LICENSEE'S SPOUSE AT THE TIME OF THE CONTACT OR,
33 IMMEDIATELY PRECEDING THE NURSE-PATIENT RELATIONSHIP, WAS IN A DATING OR
34 ENGAGEMENT RELATIONSHIP WITH THE LICENSEE. FOR THE PURPOSES OF THIS
35 SUBDIVISION, "SEXUAL CONDUCT" INCLUDES:
- 36 (i) ENGAGING IN OR SOLICITING A SEXUAL RELATIONSHIP, WHETHER
37 CONSENSUAL OR NONCONSENSUAL.
- 38 (ii) MAKING SEXUAL ADVANCES, REQUESTING SEXUAL FAVORS OR ENGAGING
39 IN ANY OTHER VERBAL CONDUCT OR PHYSICAL CONTACT OF A SEXUAL NATURE.
- 40 (iii) INTENTIONALLY VIEWING A COMPLETELY OR PARTIALLY DISROBED
41 PATIENT IN THE COURSE OF TREATMENT IF THE VIEWING IS NOT RELATED TO
42 PATIENT DIAGNOSIS OR TREATMENT UNDER CURRENT PRACTICE STANDARDS.

1 Sec. 2. Section 32-1606, Arizona Revised Statutes, is amended to
2 read:

3 32-1606. Powers and duties of board

4 A. The board may:

5 1. Adopt and revise rules necessary to carry into effect this
6 chapter.

7 2. Publish advisory opinions regarding registered and practical
8 nursing practice and nursing education.

9 3. Issue limited licenses or certificates if it determines that an
10 applicant or licensee cannot function safely in a specific setting or
11 within the full scope of practice.

12 4. Refer criminal violations of this chapter to the appropriate law
13 enforcement agency.

14 5. Establish a confidential program for monitoring licensees who
15 are chemically dependent and who enroll in rehabilitation programs that
16 meet the criteria established by the board. The board may take further
17 action if the licensee refuses to enter into a stipulated agreement or
18 fails to comply with its terms. In order to protect the public health and
19 safety, the confidentiality requirements of this paragraph do not apply if
20 the licensee does not comply with the stipulated agreement.

21 6. On the applicant's or regulated party's request, establish a
22 payment schedule with the applicant or regulated party.

23 7. Provide education regarding board functions.

24 8. Collect or assist in collecting workforce data.

25 9. Adopt rules to conduct pilot programs consistent with public
26 safety for innovative applications in nursing practice, education and
27 regulation.

28 10. Grant retirement status on request to retired nurses who are or
29 were licensed under this chapter, who have no open complaint or
30 investigation pending against them and who are not subject to discipline.

31 11. Accept and spend federal monies and private grants, gifts,
32 contributions and devises to assist in carrying out the purposes of this
33 chapter. These monies do not revert to the state general fund at the end
34 of the fiscal year.

35 B. The board shall:

36 1. Approve regulated training and educational programs that meet
37 the requirements of this chapter and rules adopted by the board.

38 2. By rule, establish approval and reapproval processes for nursing
39 and nursing assistant training programs that meet the requirements of this
40 chapter and board rules.

41 3. Prepare and maintain a list of approved nursing programs to
42 prepare registered nurses and practical nurses whose graduates are
43 eligible for licensing under this chapter as registered nurses or as
44 practical nurses if they satisfy the other requirements of this chapter
45 and board rules.

46 4. Examine qualified registered nurse and practical nurse
47 applicants.

1 5. License and renew the licenses of qualified registered nurse and
2 practical nurse applicants and licensed nursing assistants who are not
3 qualified to be licensed by the executive director.
4 6. Adopt a seal, which the executive director shall keep.
5 7. Keep a record of all proceedings.
6 8. For proper cause, deny or rescind approval of a regulated
7 training or educational program for failure to comply with this chapter or
8 the rules of the board.
9 9. Adopt rules to approve credential evaluation services that
10 evaluate the qualifications of applicants who graduated from an
11 international nursing program.
12 10. Determine and administer appropriate disciplinary action
13 against all regulated parties who are found guilty of violating this
14 chapter or rules adopted by the board.
15 11. Perform functions necessary to carry out the requirements of
16 the nursing assistant and nurse aide training and competency evaluation
17 program as set forth in the omnibus budget reconciliation act of 1987
18 (P.L. 100-203; 101 Stat. 1330), as amended by the medicare catastrophic
19 coverage act of 1988 (P.L. 100-360; 102 Stat. 683). These functions shall
20 include:
21 (a) Testing and registering certified nursing assistants.
22 (b) Testing and licensing licensed nursing assistants.
23 (c) Maintaining a list of board-approved training programs.
24 (d) Maintaining a registry of nursing assistants for all certified
25 nursing assistants and licensed nursing assistants.
26 (e) Assessing fees.
27 12. Adopt rules establishing acts that may be performed by a
28 registered nurse practitioner or certified nurse midwife, except that the
29 board does not have authority to decide scope of practice relating to
30 abortion as defined in section 36-2151.
31 13. Adopt rules that prohibit registered nurse practitioners,
32 clinical nurse specialists or certified nurse midwives from dispensing a
33 schedule II controlled substance that is an opioid, except for an
34 implantable device or an opioid that is for medication-assisted treatment
35 for substance use disorders or as provided in section 32-3248.03.
36 14. Adopt rules establishing educational requirements to certify
37 school nurses.
38 15. Publish copies of board rules and distribute these copies on
39 request.
40 16. Require each applicant for initial licensure or certification
41 to submit a full set of fingerprints to the board for the purpose of
42 obtaining a state and federal criminal records check pursuant to section
43 41-1750 and Public Law 92-544. The department of public safety may
44 exchange this fingerprint data with the federal bureau of investigation.
45 17. Except for a licensee who has been convicted of a felony that
46 has been designated a misdemeanor pursuant to section 13-604, revoke a
47 license of a person, revoke the multistate licensure privilege of a person

1 pursuant to section 32-1669 or not issue a license or renewal to an
2 applicant who has one or more felony convictions and who has not received
3 an absolute discharge from the sentences for all felony convictions three
4 or more years before the date of filing an application pursuant to this
5 chapter.

6 18. Establish standards to approve and reapprove registered nurse
7 practitioner and clinical nurse specialist programs and provide for
8 surveys of registered nurse practitioner and clinical nurse specialist
9 programs as the board deems necessary.

10 19. Provide the licensing authorities of health care institutions,
11 facilities and homes with any information the board receives regarding
12 practices that place a patient's health at risk.

13 20. Limit the multistate licensure privilege of any person who
14 holds or applies for a license in this state pursuant to section 32-1668.

15 21. Adopt rules to establish competency standards for obtaining and
16 maintaining a license.

17 22. Adopt rules to qualify and certify clinical nurse specialists.

18 23. Adopt rules to approve and reapprove refresher courses for
19 nurses who are not currently practicing.

20 24. Maintain a list of approved medication assistant training
21 programs.

22 25. Test and certify medication assistants.

23 26. Maintain a registry and disciplinary record of medication
24 assistants who are certified pursuant to this chapter.

25 27. Adopt rules to establish the requirements for a clinical nurse
26 specialist to prescribe and dispense drugs and devices consistent with
27 section 32-1651 and within the clinical nurse specialist's population or
28 disease focus.

29 28. Issue registrations to administer general anesthesia and
30 sedation in dental offices and dental clinics pursuant to section 32-1272
31 to certified registered nurse anesthetists who have national board
32 certification in anesthesiology.

33 29. POST ALL ~~WRITTEN~~ SUBSTANTIVE ~~POLICIES~~ [POLICY STATEMENTS AS
34 DEFINED IN SECTION 41-1001] IN A CLEARLY IDENTIFIABLE SECTION ON THE
35 BOARD'S PUBLIC WEBSITE [AS REQUIRED BY SECTION 41-1091.01].

36 C. The board may conduct an investigation on receipt of information
37 that indicates that a person or regulated party may have violated this
38 chapter or a rule adopted pursuant to this chapter. Following the
39 investigation, the board may take disciplinary action pursuant to this
40 chapter.

41 D. The board may limit, revoke or suspend the privilege of a nurse
42 to practice in this state granted pursuant to section 32-1668.

43 E. Failure to comply with any final order of the board, including
44 an order of censure or probation, is cause for suspension or revocation of
45 a license or a certificate.

46 F. The president or a member of the board designated by the
47 president may administer oaths in transacting the business of the board.

1 Sec. 3. Section 32-1644, Arizona Revised Statutes, is amended to
2 read:

3 32-1644. Approval of nursing schools and nursing programs:
4 application; maintenance of standards

5 A. The board shall approve all new prelicensure nursing, REGISTERED
6 nurse practitioner and clinical nurse specialist programs pursuant to this
7 section. A postsecondary educational institution or school in this state
8 that is accredited by an accrediting agency recognized by the United
9 States department of education desiring to conduct a registered nursing,
10 practical nursing, REGISTERED nurse practitioner or clinical nurse
11 specialist program shall apply to the board for approval and submit
12 satisfactory proof that ~~it~~ THE INSTITUTION OR SCHOOL is prepared to meet
13 and maintain the minimum standards prescribed by this chapter and board
14 rules.

15 B. The board or its authorized agent shall conduct a survey of the
16 institution or program applying for approval and shall submit a written
17 report of its findings to the board. If the board determines that the
18 program meets the requirements prescribed in its rules, ~~it~~ THE BOARD shall
19 approve the applicant as either a registered nursing program, practical
20 nursing program, REGISTERED nurse practitioner program or clinical nurse
21 specialist program in a specialty area.

22 C. A nursing program approved by the board may also be accredited
23 by a national nursing accrediting agency recognized by the board. If a
24 prelicensure nursing program is accredited by a national nursing
25 accrediting agency recognized by the board, the board does not have
26 authority over ~~it~~ THE PROGRAM unless any of the following occurs:

27 1. The board receives a complaint about the program relating to
28 patient safety.

29 [2. The program falls] [AND REMAINS] [below the standards
30 prescribed by the board in its rules] [FOR ANNUAL NATIONAL COUNCIL
31 LICENSURE EXAMINATION PASS RATES][.]

32 ~~[2.]~~ [3.] The program loses its accreditation by a national
33 nursing accrediting agency recognized by the board.

34 ~~[3.]~~ [4.] The program allows its accreditation by a national
35 nursing accrediting agency recognized by the board to lapse.

36 D. From time to time the board, through its authorized employees or
37 representatives, may resurvey all approved programs in ~~the~~ THIS state and
38 shall file written reports of these resurveys with the board. If the
39 board determines that an approved nursing program is not maintaining the
40 required standards, ~~it~~ THE BOARD shall ~~immediately~~ give written notice to
41 the program specifying the defects. If the defects are not corrected
42 within ~~a reasonable~~ THE time ~~as determined~~ PRESCRIBED by the board BY
43 RULE, the board may take either of the following actions:

44 1. Approve the program but restrict the program's ability to admit
45 new students until the program complies with board standards.

46 2. Remove the program from the list of approved nursing programs
47 until the program complies with board standards.

1 E. All approved nursing programs shall maintain accurate and
2 current records showing in full the theoretical and practical courses
3 given to each student.

4 F. The board does not have regulatory authority over the following
5 approved REGISTERED nurse practitioner or clinical nurse specialist
6 programs unless the conditions prescribed in subsection C OF THIS SECTION
7 are met:

8 1. A REGISTERED nurse practitioner or clinical nurse specialist
9 program that is part of a graduate program in nursing accredited by an
10 agency recognized by the board if the program was surveyed as part of the
11 graduate program accreditation.

12 2. A REGISTERED nurse practitioner or clinical nurse specialist
13 program that is accredited by an agency recognized by the board.

14 [G. THE ARIZONA STATE BOARD OF NURSING SHALL SHARE, WITHIN FIVE
15 BUSINESS DAYS, ANY COMPLAINT IT RECEIVES AGAINST A NURSING PROGRAM OR
16 NURSING SCHOOL WITH THE STATE BOARD FOR PRIVATE POSTSECONDARY EDUCATION.
17 THE ARIZONA BOARD OF REGENTS OR THE COMMUNITY COLLEGE DISTRICT GOVERNING
18 BOARD, AS APPLICABLE.]

19 Sec. 4. Section 32-1664, Arizona Revised Statutes, is amended to
20 read:

21 32-1664. Investigation; hearing; notice

22 A. In connection with an investigation, the board or its duly
23 authorized agents or employees may obtain any documents, reports, records,
24 papers, books and materials, including hospital records, medical staff
25 records and medical staff review committee records, or any other physical
26 evidence that indicates that a person or regulated party may have violated
27 this chapter or a rule adopted pursuant to this chapter:

28 1. By entering the premises, at any reasonable time, and inspecting
29 and copying materials in the possession of a regulated party that relate
30 to nursing competence, unprofessional conduct or the mental or physical
31 ability of a licensee to safely practice nursing.

32 2. By issuing a subpoena under the board's seal to require the
33 attendance and testimony of witnesses or to demand the production for
34 examination or copying of documents or any other physical evidence. Within
35 five days after a person is served with a subpoena, that person may
36 petition the board to revoke, limit or modify the subpoena. The board
37 shall do so if in its opinion the evidence required does not relate to
38 unlawful practices covered by this chapter, is not relevant to the charge
39 that is the subject matter of the hearing or investigation or does not
40 describe with sufficient particularity the physical evidence whose
41 production is required.

42 3. By submitting a written request for the information.

43 4. In the case of an applicant's or a regulated party's personal
44 medical records, as defined in section 12-2291, by any means allowed by
45 this section if the board either:

46 (a) Obtains from the applicant or regulated party, or the health
47 care decision maker of the applicant or regulated party, a written

1 authorization that satisfies the requirements of title 12, chapter 13,
2 article 7.1.

3 (b) Reasonably believes that the records relate to information
4 already in the board's possession regarding the competence, unprofessional
5 conduct or mental or physical ability of the applicant or regulated party
6 as it pertains to safe practice. If the board adopts a substantive policy
7 statement pursuant to section 41-1091, it may authorize the executive
8 director, or a designee in the absence of the executive director, to make
9 the determination of reasonable belief.

10 B. A regulated party and a health care institution as defined in
11 section 36-401 shall, and any other person may, report to the board any
12 information the licensee, certificate holder, health care institution or
13 individual may have that appears to show that a regulated party or
14 applicant is, was or may be a threat to the public health or safety.

15 C. If a death or an incident requiring emergency medical response
16 occurs in a dental office or dental clinic during the administration of or
17 recovery from general anesthesia or sedation by a certified registered
18 nurse anesthetist, the certified registered nurse anesthetist shall, and
19 any other person may, report the death or incident to the board within
20 seven business days after the occurrence.

21 D. The board retains jurisdiction to proceed with an investigation
22 or a disciplinary proceeding against a regulated party whose license or
23 certificate expired not more than five years before the board initiates
24 the investigation.

25 E. Any regulated party, health care institution or other person
26 that reports or provides information to the board in good faith is not
27 subject to civil liability. If requested the board shall not disclose the
28 name of the reporter unless the information is essential to proceedings
29 conducted pursuant to this section.

30 F. Any regulated party or person who is subject to an investigation
31 may obtain representation by counsel.

32 G. On determination of reasonable cause, the board, or if delegated
33 by the board the executive director, may require a licensee, certificate
34 holder or applicant to undergo at the expense of the licensee, certificate
35 holder or applicant any combination of mental, physical or psychological
36 examinations, assessments or skills evaluations necessary to determine the
37 person's competence or ability to practice safely. These examinations may
38 include bodily fluid testing and other examinations known to detect the
39 presence of alcohol or drugs. If the executive director orders the
40 licensee, applicant or certificate holder to undertake an examination,
41 assessment or evaluation pursuant to this subsection, and the licensee,
42 certificate holder or applicant fails to affirm to the board in writing
43 within fifteen days after receipt of the notice of the order that the
44 licensee, certificate holder or applicant intends to comply with the
45 order, the executive director shall refer the matter to the board to allow
46 the board to determine whether to issue an order pursuant to this
47 subsection. At each regular meeting of the board the executive director

1 shall report to the board data concerning orders issued by the executive
2 director pursuant to this subsection since the last regular meeting of the
3 board and any other data requested by the board.

4 H. The board shall provide the investigative report if requested
5 pursuant to section 32-3206.

6 I. If after completing its investigation the board finds that the
7 information provided pursuant to this section is not of sufficient
8 seriousness to merit disciplinary action against the regulated party or
9 applicant, it may take either of the following actions:

10 1. Dismiss if in the opinion of the board the information is
11 without merit.

12 2. File a letter of concern if in the opinion of the board there is
13 insufficient evidence to support disciplinary action against the regulated
14 party or applicant but sufficient evidence for the board to notify the
15 regulated party or applicant of its concern.

16 J. Except as provided pursuant to section 32-1663, subsection F and
17 subsection K of this section, if the investigation in the opinion of the
18 board reveals reasonable grounds to support the charge, the regulated
19 party is entitled to an administrative hearing pursuant to title 41,
20 chapter 6, article 10. If notice of the hearing is served by certified
21 mail, service is complete on the date the notice is placed in the mail.

22 K. A regulated party shall respond in writing to the board within
23 thirty days after notice of the hearing is served as prescribed in
24 subsection J of this section. The board may consider a regulated party's
25 failure to respond within this time as an admission by default to the
26 allegations stated in the complaint. The board may then take disciplinary
27 actions allowed by this chapter without conducting a hearing.

28 L. An administrative law judge or a panel of board members may
29 conduct hearings pursuant to this section.

30 M. In any matters pending before it, the board may issue subpoenas
31 under its seal to compel the attendance of witnesses.

32 N. Patient records, including clinical records, medical reports,
33 laboratory statements and reports, any file, film, other report or oral
34 statement relating to diagnostic findings or treatment of patients, any
35 information from which a patient or a patient's family might be identified
36 or information received and records kept by the board as a result of the
37 investigation procedure outlined in this chapter are not available to the
38 public and are not subject to discovery in civil or criminal proceedings.

39 O. Hospital records, medical staff records, medical staff review
40 committee records, testimony concerning these records and proceedings
41 related to the creation of these records shall not be available to the
42 public. They shall be kept confidential by the board and shall be subject
43 to the same provisions concerning discovery and use in legal actions as
44 are the original records in the possession and control of hospitals, their
45 medical staffs and their medical staff review committees. The board shall
46 use these records and testimony during the course of investigations and
47 proceedings pursuant to this chapter.

1 P. If the regulated party is found to have committed an act of
2 unprofessional conduct or to have violated this chapter or a rule adopted
3 pursuant to this chapter, the board may take disciplinary action.

4 Q. The board may subsequently issue a denied license or certificate
5 and may reissue a revoked or voluntarily surrendered license or
6 certificate.

7 R. On application by the board to any superior court judge, a
8 person who without just cause fails to comply with a subpoena issued
9 pursuant to this section may be ordered by the judge to comply with the
10 subpoena and punished by the court for failing to comply. Subpoenas shall
11 be served by regular or certified mail or in the manner required by the
12 Arizona rules of civil procedure.

13 S. The board may share investigative information that is
14 confidential under subsections N and O of this section with other state,
15 federal and international health care agencies and with state, federal and
16 international law enforcement authorities if the recipient is subject to
17 confidentiality requirements similar to those established by this section.
18 A disclosure made by the board pursuant to this subsection is not a waiver
19 of the confidentiality requirements established by this section.

20 T. THE ARIZONA STATE BOARD OF NURSING SHALL TRANSFER, WITHIN FIVE
21 BUSINESS DAYS, ANY COMPLAINT IT RECEIVES AGAINST A NURSING PROGRAM OR
22 NURSING SCHOOL TO THE STATE BOARD FOR PRIVATE POSTSECONDARY EDUCATION, THE
23 ARIZONA BOARD OF REGENTS OR THE COMMUNITY COLLEGE DISTRICT GOVERNING
24 BOARD, AS APPLICABLE.

25 Sec. 5. Title 32, chapter 15, article 3, Arizona Revised Statutes,
26 is amended by adding sections 32-1664.01, 32-1664.02, 32-1664.03,
27 32-1664.04 and 32-1664.05, to read:

28 32-1664.01. Investigation of complaints; confidentiality;
29 limits; notice; evaluations; timeframe;
30 definition

31 A. NOTWITHSTANDING ANY OTHER PROVISION OF THIS CHAPTER OR CHAPTER
32 32 OF THIS TITLE TO THE CONTRARY:

33 1. EXCEPT AS SET FORTH IN PARAGRAPH 3 OF THIS SUBSECTION, THE BOARD
34 SHALL REQUIRE COMPLAINANTS TO IDENTIFY THEMSELVES IN THE COMPLAINT AND
35 MAKE THEMSELVES AVAILABLE FOR AN EVIDENTIARY INTERVIEW. COMPLAINANTS MAY
36 REQUEST THAT THEIR IDENTITY REMAIN CONFIDENTIAL DURING THE INVESTIGATORY
37 PROCESS. IF THE INVESTIGATORY PROCESS RESULTS IN A DETERMINATION THAT A
38 VIOLATION OF LAW MAY HAVE OCCURRED, THE RESPONDENT IS ENTITLED TO REVIEW
39 THE COMPLETE INVESTIGATORY FILE, INCLUDING THE IDENTITY OF THE COMPLAINANT
40 EXCEPT AS PROVIDED IN PARAGRAPH 2 OF THIS SUBSECTION.

41 2. THE BOARD, [SOLELY AT THE BOARD'S DISCRETION,] ON FINDING CAUSE
42 THAT A COMPLAINANT MAY REASONABLY FEAR RETALIATION OR BE ENDANGERED IF THE
43 COMPLAINANT'S IDENTITY IS REVEALED OR IF THE COMPLAINT DIRECTLY IMPACTS
44 PATIENT SAFETY, MAY CONTINUE TO MAINTAIN THE COMPLAINANT'S CONFIDENTIALITY
45 FROM THE LICENSEE UNTIL THE CONCLUSION OF THE [INVESTIGATIVE AND]
46 ADMINISTRATIVE PROCESS. THE BOARD MAY CONDUCT A CLOSED EVIDENTIARY
47 HEARING IF THE COMPLAINANT REQUESTS THAT THE COMPLAINANT'S IDENTITY REMAIN

1 PRIVATE AND THE BOARD HAS A REASONABLE BASIS TO CONDUCT THE HEARING. A
2 COMPLAINANT'S ANONYMITY MAY CONTINUE UNTIL EVIDENCE BY THE COMPLAINANT IS
3 REQUIRED AT AN ADMINISTRATIVE PROCEEDING PURSUANT TO TITLE 41 OR A LEGAL
4 PROCEEDING.

5 3. THE BOARD MAY TAKE ACTION ON A COMPLAINT IF THE COMPLAINANT
6 REFUSES TO IDENTIFY HIMSELF OR HERSELF ONLY IF THE BOARD HAS SUFFICIENT
7 INFORMATION THAT A VIOLATION MAY HAVE OCCURRED WITHIN ITS JURISDICTION
8 THAT DIRECTLY IMPACTS THE SAFETY OF PATIENTS WITHOUT THE TESTIMONY OF THE
9 ANONYMOUS COMPLAINANT. [THE BOARD MAY ATTEMPT TO SUBSTANTIATE AN
10 ANONYMOUS COMPLAINT THROUGH FURTHER INVESTIGATION PURSUANT TO THE BOARD'S
11 POLICIES AND MAY OPEN A COMPLAINT AGAINST A LICENSEE OR CERTIFICATE HOLDER
12 IF THE ANONYMOUS COMPLAINT IS SUBSTANTIATED THROUGH CORROBORATION FROM
13 RECORDS, OTHER WITNESSES, PATTERN EVIDENCE OR DATA SOURCES.]

14 4. THE BOARD SHALL LIMIT AN INVESTIGATION OF A COMPLAINT TO THOSE
15 INVESTIGATIVE SUBJECTS AND ACTIONS THAT ARE RELEVANT AND MATERIAL TO THE
16 ISSUES RAISED IN THE COMPLAINT AND THAT WOULD REASONABLY BE TAKEN TO
17 INVESTIGATE THE ISSUES IN THE COMPLAINT. THIS PARAGRAPH DOES NOT PROHIBIT
18 THE BOARD FROM INVESTIGATING ANY ISSUE OR EVIDENCE OF UNPROFESSIONAL
19 CONDUCT THAT IS DISCOVERED OR BROUGHT TO THE ATTENTION OF THE BOARD DURING
20 THE INVESTIGATION OF THE ORIGINAL COMPLAINT.

21 5. ON REASONABLE BELIEF THAT A CRIME HAS BEEN COMMITTED, THE BOARD
22 SHALL REPORT THE ALLEGED CRIMINAL CONDUCT TO THE APPROPRIATE CRIMINAL
23 JUSTICE AGENCY. IF THE BOARD HAS A REASONABLE BELIEF THAT CONDUCT BY A
24 LICENSED, PERMITTED OR CERTIFICATED INDIVIDUAL OR OTHER ENTITY OVER WHICH
25 THE BOARD DOES NOT HAVE JURISDICTION MAY VIOLATE THE LAW OR CODES OF
26 CONDUCT, THE BOARD SHALL REPORT THE CONDUCT TO THE APPROPRIATE STATE
27 REGULATORY BOARD OR STATE AGENCY THAT THE BOARD REASONABLY BELIEVES HAS
28 JURISDICTION OVER THE LICENSEE, PERMITTEE OR CERTIFICATE HOLDER OR OTHER
29 ENTITY.

30 6. THE BOARD SHALL IMPLEMENT A POLICY PRIORITIZING COMPLAINTS BASED
31 ON THE HARM TO A PATIENT OR POTENTIALLY TO THE PUBLIC. THE BOARD SHALL
32 ASSIGN THE HIGHEST PRIORITY TO COMPLAINTS ALLEGING SEXUAL MISCONDUCT WITH
33 A PATIENT, [ABUSE OR NEGLECT OF A PATIENT, PRACTICE BEYOND THE SCOPE OF
34 PRACTICE THAT CREATED A SIGNIFICANT RISK OF HARM.] CRIMINAL ASSAULT OR
35 THEFT OR PROVIDING SERVICES WHILE UNDER THE INFLUENCE OF ANY ILLEGAL OR
36 LEGAL SUBSTANCE THAT IMPAIRS THE LICENSEE OR CERTIFICATE HOLDER.

37 7. THE BOARD SHALL PROVIDE THE RESPONDENT WITH A WRITTEN NOTICE
38 STATING THAT THERE IS AN OPEN INVESTIGATION, THE SUBSTANCE OF THE
39 COMPLAINT, THAT THE RESPONDENT HAS THE RIGHT TO BE REPRESENTED BY LEGAL
40 COUNSEL AND THAT THE RESPONDENT HAS AT LEAST FIFTEEN BUSINESS DAYS AFTER
41 RECEIVING THE WRITTEN NOTICE BEFORE THE BOARD REQUIRES A RESPONSE[, EXCEPT
42 THAT THE BOARD MAY REQUIRE AN IMMEDIATE RESPONSE OR PRODUCTION OF RECORDS
43 WHEN NECESSARY TO PROTECT PUBLIC HEALTH AND SAFETY]. THE WRITTEN NOTICE
44 SHALL ALSO INFORM THE RESPONDENT THAT THE BOARD MAY USE ANY STATEMENT THE
45 RESPONDENT MAKES AGAINST THE RESPONDENT. THE RESPONDENT MAY WAIVE THE
46 FIFTEEN-DAY TIME FRAME BY WRITTEN COMMUNICATION.

1 8. IF THE BOARD DETERMINES THAT A PSYCHOLOGICAL, PSYCHIATRIC OR
2 OTHER MEDICAL EVALUATION OF THE LICENSEE OR CERTIFICATE HOLDER IS
3 ESSENTIAL FOR THE BOARD TO MAKE A DECISION REGARDING A COMPLAINT AND
4 ORDERS THE LICENSEE OR CERTIFICATE HOLDER TO OBTAIN AN EVALUATION, AND THE
5 LICENSEE OR CERTIFICATE HOLDER REQUESTS THAT THE EVALUATION BE MADE BY A
6 PROFESSIONAL OTHER THAN THE PROFESSIONAL RECOMMENDED BY THE BOARD, THE
7 BOARD OR ITS DESIGNEE [SHALL CONSIDER THE ALTERNATIVE EVALUATOR'S
8 CREDENTIALS, INDEPENDENCE AND CONFLICT DISCLOSURE AND] MAY ACCEPT AND
9 APPROVE AN EVALUATION FROM A PROFESSIONAL WHO HAS THE CREDENTIALS,
10 TRAINING, EXPERTISE AND IMPARTIALITY REQUIRED TO ADDRESS THE ISSUES THE
11 BOARD HAS REQUESTED IN ITS ORDER. ~~[THE BOARD MAY NOT REQUIRE THE LICENSEE~~
12 ~~OR CERTIFICATE HOLDER TO BE EVALUATED ONLY BY A PROFESSIONAL WHOSE NAME IS~~
13 ~~PROVIDED IN A LIST BY THE BOARD TO THE LICENSEE OR CERTIFICATE HOLDER.]~~

14 9. WITHIN ONE HUNDRED EIGHTY DAYS AFTER THE BOARD RECEIVES A
15 COMPLAINT AGAINST A LICENSEE OR CERTIFICATE HOLDER, THE BOARD SHALL DO ONE
16 OF THE FOLLOWING:

17 (a) SUBMIT THE INVESTIGATION FOR REVIEW.

18 (b) ADMINISTRATIVELY DISMISS THE COMPLAINT IF THE BOARD FINDS IT IS
19 UNSUBSTANTIATED.

(c) REPORT A DETERMINATION EITHER THAT THE COMPLAINT INVESTIGATION CANNOT BE REASONABLY COMPLETED WITHIN ONE HUNDRED EIGHTY DAYS DUE TO THE COMPLEXITY OF THE MATTER, THE RESPONDENT'S REQUEST FOR ADDITIONAL TIME TO RESPOND OR CAUSING DELAYS IN THE INVESTIGATION OR THE FAILURE OF A PERSON OR ENTITY TO TIMELY RESPOND TO A BOARD SUBPOENA. IF THIS DETERMINATION IS MADE, THE BOARD, WITHIN THE ONE HUNDRED EIGHTY-DAY PERIOD, SHALL TAKE ONE OF THE FOLLOWING ACTIONS:

27 (i) CONTINUE THE INVESTIGATION FOR AN ADDITIONAL ONE HUNDRED DAYS
28 TO COMPLETE THE INVESTIGATION AND PROCEED WITH THE ADMINISTRATIVE
29 PROCEDURE TO SUBMIT THE COMPLAINT FOR FINAL BOARD REVIEW AND ACTION.

(ii) ADMINISTRATIVELY DISMISS THE COMPLAINT WITHOUT PREJUDICE. IF THE BOARD ADMINISTRATIVELY DISMISSES THE COMPLAINT WITHOUT PREJUDICE, THE BOARD MAY REOPEN THE INVESTIGATION ONLY IF THE BOARD RECEIVES ADDITIONAL EVIDENCE, INFORMATION OR TESTIMONY SUFFICIENT TO CONCLUDE THE INVESTIGATION WITHIN TWO YEARS AFTER THE ADMINISTRATIVE DISMISSAL AS DESCRIBED IN THIS ITEM.

36 10. IF ANOTHER HEALTH PROFESSION REGULATORY BOARD OR A CRIMINAL
37 JUSTICE AGENCY IS INVESTIGATING THE CIRCUMSTANCES ALLEGED IN A COMPLAINT,
38 THE TIME FRAMES PRESCRIBED IN PARAGRAPH 9 OF THIS SUBSECTION ARE SUSPENDED
39 UNTIL THE INVESTIGATION IS COMPLETED.

40 B. FOR THE PURPOSES OF THIS SECTION, "WITHOUT PREJUDICE" MEANS THAT
41 THE BOARD MAY OPEN ANOTHER COMPLAINT BASED ON THE SAME SET OF FACTS OF A
42 COMPLAINT THAT HAS BEEN DISMISSED IF ADDITIONAL EVIDENCE OR INFORMATION
43 BECOMES AVAILABLE TO SUBSTANTIATE THE COMPLAINT[. INCLUDING NEW
44 WITNESSES].

45 32-1664.02. Investigative files; access; confidentiality;
46 definition

1 A. NOTWITHSTANDING ANY PROVISION OF THIS CHAPTER OR CHAPTER 32 OF
2 THIS TITLE TO THE CONTRARY:

3 1. THE BOARD SHALL PROVIDE NOTICE TO A RESPONDENT AT LEAST FIFTEEN
4 BUSINESS DAYS BEFORE A MEETING OF THE BOARD TO REVIEW THE STATUS OF AN
5 INVESTIGATION. THE BOARD SHALL PROVIDE THE RESPONDENT WITH ACCESS TO THE
6 INVESTIGATIVE FILE BY EITHER OF THE FOLLOWING METHODS AT THE DISCRETION OF
7 THE RESPONDENT:

8 (a) ALLOW THE RESPONDENT TO REVIEW THE FILE AT THE BOARD OFFICE AND
9 RECEIVE COPIES.

10 (b) PROVIDE THE FILE TO THE RESPONDENT OR THE RESPONDENT'S ATTORNEY
11 BY ELECTRONIC TRANSMISSION.

12 2. A PERSON WHO OBTAINS INFORMATION FROM THE BOARD PURSUANT TO THIS
13 SECTION MAY NOT RELEASE THE INFORMATION TO ANY OTHER PERSON OR ENTITY OR
14 USE THE INFORMATION IN ANY PROCEEDING OR ACTION EXCEPT IN CONNECTION WITH
15 THE BOARD'S REVIEW OF THE INVESTIGATION, THE DISCIPLINARY INTERVIEW AND
16 ANY ADMINISTRATIVE PROCEEDING OR APPEAL RELATED TO THE DISCIPLINARY
17 INTERVIEW OR HEARING. A PERSON WHO VIOLATES THIS PARAGRAPH COMMITS AN ACT
18 OF UNPROFESSIONAL CONDUCT.

19 3. THE BOARD MAY NOT MAKE AN ADMINISTRATIVE DISMISSAL OR
20 NONDISCIPLINARY REMEDIAL ACTION PUBLICLY AVAILABLE OR REPORT THE ACTION TO
21 THE NATIONAL PRACTITIONER DATA BANK, CONSISTENT WITH FEDERAL LAW.

22 B. FOR THE PURPOSES OF THIS SECTION, "INVESTIGATIVE FILE" INCLUDES:

23 1. ANY WITNESS OR COMPLAINANT STATEMENT AND ANY DOCUMENTARY
24 EVIDENCE OBTAINED DURING THE INVESTIGATION.

25 2. ANY REVIEW CONDUCTED BY AN EXPERT OR CONSULTANT PROVIDING AN
26 EVALUATION OF OR OPINION ON THE ALLEGATIONS.

27 3. ANY RECORDS ON THE PATIENT OBTAINED BY THE BOARD FROM OTHER
28 HEALTH CARE PROVIDERS.

29 4. THE RESULTS OF ANY EVALUATION OR TEST OF THE LICENSEE OR
30 CERTIFICATE HOLDER CONDUCTED AT THE BOARD'S DIRECTION.

31 5. ANY OTHER FACTUAL INFORMATION THAT THE BOARD WILL USE IN MAKING
32 ITS DETERMINATION.

33 6. ANY RECOMMENDATION TO THE BOARD FOR THE DISPOSITION OF THE
34 INVESTIGATION.

35 32-1664.03. Burden of proof

36 EXCEPT FOR DISCIPLINARY MATTERS BROUGHT PURSUANT TO SECTION 32-1601,
37 PARAGRAPH 27, SUBDIVISION (n), THE BOARD HAS THE BURDEN OF PROOF BY CLEAR
38 AND CONVINCING EVIDENCE FOR DISCIPLINARY MATTERS BROUGHT PURSUANT TO THIS
39 CHAPTER.

40 32-1664.04. Disciplinary actions; expungement; exceptions

41 NOTWITHSTANDING ANY OTHER PROVISION OF THIS CHAPTER TO THE CONTRARY:

42 1. THE BOARD[. SOLELY AT THE BOARD'S DISCRETION.] MAY GRANT A
43 REQUEST FOR EXPUNGEMENT OF A DISCIPLINARY ACTION PREVIOUSLY IMPOSED
44 AGAINST A LICENSEE OR CERTIFICATE HOLDER, WHETHER FORMAL OR INFORMAL, ONLY
45 AS AUTHORIZED BY THIS SECTION. THE BOARD MAY GRANT AN EXPUNGEMENT OF A
46 DISCIPLINARY ACTION ONLY ONCE FOR A LICENSEE OR CERTIFICATE HOLDER.

1 2. ANY REQUEST FOR EXPUNGEMENT PURSUANT TO THIS SECTION SHALL BE
2 MADE IN WRITING AND SHALL COMPLY WITH THIS SECTION.

3 3. ANY DISCIPLINARY ACTION ARISING FROM THE FAILURE TO TIMELY RENEW
4 A LICENSE OR CERTIFICATE, THE FAILURE TO COMPLETE CONTINUING EDUCATION OR
5 A DOCUMENTATION ERROR SHALL BE EXPUNGED BY THE BOARD IF ALL OF THE
6 FOLLOWING APPLY:

7 (a) THE DISCIPLINARY ACTION IS AT LEAST TWO YEARS OLD.

8 (b) THE TERMS OF THE DISCIPLINARY ACTION HAVE BEEN MET.

9 (c) THERE HAVE BEEN NO SUBSEQUENT VIOLATIONS OF ANY OTHER
10 PROVISIONS OF THIS CHAPTER OR THE RULES ADOPTED PURSUANT TO THIS CHAPTER.

11 4. EXCEPT AS PROVIDED IN PARAGRAPH 5 OF THIS SECTION, FOR ANY OTHER
12 DISCIPLINARY ACTION NOT IDENTIFIED IN PARAGRAPH 3 OF THIS SECTION, THE
13 BOARD MAY EXPUNGE A DISCIPLINARY ACTION IF ALL OF THE FOLLOWING APPLY:

14 (a) THE DISCIPLINARY ACTION AT ISSUE IS AT LEAST FIVE YEARS OLD.

15 (b) THE TERMS OF THE DISCIPLINARY ACTION HAVE BEEN MET.

16 (c) THERE HAVE BEEN NO SUBSEQUENT VIOLATIONS OF ANY OTHER
17 PROVISIONS OF THIS CHAPTER OR THE RULES ADOPTED PURSUANT TO THIS CHAPTER.

18 (d) THE LICENSEE OR CERTIFICATE HOLDER HAS HAD A TOTAL OF NOT MORE
19 THAN TWO DISCIPLINARY ACTIONS IMPOSED, INCLUDING THE DISCIPLINARY ACTION
20 FOR WHICH EXPUNGEMENT IS REQUESTED, BY THE BOARD OR ANY OTHER REGULATORY
21 BOARD OR AUTHORITY FROM THIS STATE OR ANY OTHER STATE.

22 5. THE BOARD MAY NOT GRANT A REQUEST FOR EXPUNGEMENT IF ANY OF THE
23 FOLLOWING APPLIES TO THE DISCIPLINARY ACTION FOR WHICH EXPUNGEMENT IS
24 REQUESTED:

25 (a) THE DISCIPLINARY ACTION WAS BASED ON A CRIMINAL CONVICTION
26 RESULTING FROM:

27 (i) ASSAULT, ABUSE, FRAUD OR ANY OTHER FELONIOUS CONDUCT THAT
28 RESULTED IN PHYSICAL OR FINANCIAL HARM TO OR THE DEATH OF AN INDIVIDUAL.

29 (ii) ATTEMPTED MURDER, ASSAULT, ABUSE OR FRAUD.

30 (b) THE LICENSEE OR CERTIFICATE HOLDER ASSAULTED OR ABUSED A
31 PATIENT [OR THE CONDUCT RESULTED IN HARM TO A PATIENT].

32 (c) THE LICENSEE'S OR CERTIFICATE HOLDER'S CONDUCT[, INCLUDING A
33 MEDICATION OR CLINICAL PRACTICE ERROR,] CAUSED HARM OR DEATH TO A PATIENT.

34 (d) THE LICENSEE OR CERTIFICATE HOLDER COMMITTED SEXUAL MISCONDUCT
35 WITH A PATIENT.

36 (e) THE LICENSEE OR CERTIFICATE HOLDER WAS PRACTICING WHILE
37 IMPAIRED [OR DEMONSTRATED A PATTERN OF DIVERSION OF CONTROLLED
38 SUBSTANCES].

39 (f) THE LICENSEE OR CERTIFICATE HOLDER COMMITTED FRAUD OR FALSIFIED
40 [OR MALICIOUSLY ALTERED] RECORDS IN A HEALTH CARE SETTING.

41 6. THE BOARD SHALL REPORT AN EXPUNGEMENT UNDER THIS SECTION TO ANY
42 NATIONAL DATABASE TO WHICH THE BOARD PREVIOUSLY REPORTED THE DISCIPLINARY
43 ACTION.

44 7. A LICENSEE OR CERTIFICATE HOLDER IS NOT REQUIRED TO REPORT AN
45 EXPUNGED DISCIPLINARY ACTION ON ANY FUTURE APPLICATION FOR ISSUANCE OR
46 RENEWAL OF A LICENSE, PERMIT OR CERTIFICATE TO ANY REGULATORY BOARD OR
47 AGENCY IN THIS STATE.

1 ~~[S2-1664.05. Board liability; damages~~
2 ~~NOTWITHSTANDING ANY OTHER LAW, IF THE BOARD DOES NOT COMPLY WITH~~
3 ~~THIS CHAPTER, A LICENSEE OR CERTIFICATE HOLDER OR THE SUBJECT OF A BOARD~~
4 ~~INVESTIGATION WHO WAS DAMAGED BY THE BOARD'S FAILURE TO COMPLY WITH THIS~~
5 ~~CHAPTER MAY SEEK DAMAGES, INCLUDING ATTORNEY FEES, IN A CIVIL LEGAL~~
6 ~~PROCEEDING. A BOARD MEMBER OR STAFF MEMBER MAY BE HELD PERSONALLY LIABLE~~
7 ~~ONLY IF THE PERSON'S CONDUCT WAS RECKLESS, MALICIOUS OR WILFUL.]~~

8 Enroll and engross to conform

9 Amend title to conform

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